



ACCELERATING ADOPTION & USE OF AI FOR
CLINICAL CARE

HEALTH AND HUMAN SERVICES

HHS RFI 0955-AA13

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The acceleration of artificial intelligence (AI) into clinical care holds tremendous promise for improving patient outcomes, advancing public health, reducing the burdens on caregivers and providers across the healthcare landscape, and helping to eliminate fraud, waste, and abuse (FWA). Deloitte recognizes HHS's leadership in accelerating responsible adoption of AI to deliver secure and high-value health solutions for the American people, and optimizing operational spending for immediate and long-term impact. The following paper offers concrete recommendations to help HHS shape regulatory, reimbursement, and research strategies that will enable a "OneHHS" approach to AI, while upholding patient privacy, civil rights, and civil liberties.

Payment integrity is a priority for HHS, which can help patients understand payments ahead of service, increase decision speed, lower administrative burden, and increase efficiency of the payer/provider relationship. AI-supported clinical review solutions can improve the speed and consistency of Medicare payment decisions. Application of machine learning in the payment space can minimize documentation and administrative burden over time, drawing attention to the most relevant clinical data in the claims process, accelerating medical-necessity review, and easing provider burden. Using AI to accelerate the review process, while retaining human clinical oversight, ensures determinations remain clinically sound, transparent, and based on evidence. HHS could adopt flexible design features such as pre-payment review, prior authorization support, and potential compliance-based exemptions to further reduce administrative friction. Collectively, these capabilities strengthen payment integrity across HHS systems, increasing consistency and reducing waste in alignment with agency goals.²

Using AI to standardize and codify data across electronic health records (EHRs) and insurance systems will increase the speed of claims processing, support new regulations, fight FWA and improve policy implementation.⁶ Today, medical policies often live in PDFs or narrative documents that require manual interpretation, creating inconsistency, administrative burden, and audit risk for both payers and providers. AI can rapidly extract and standardize these policies, providing clear, uniform rules that accelerate medical-necessity review and reducing variation in how policies are applied. This codification also strengthens interoperability by transforming PDFs into discrete data that can flow across payer and provider systems, enabling faster, more accurate claims processing and supporting new regulatory mandates that depend on consistency, speed, and transparency. Moreover, by providing explicit guidance and guardrails for how AI should be used in medical-necessity review, HHS can give industry stakeholders confidence that automated processes are compliant, auditable, and fair, addressing long-standing fears of AI-driven denials and ensuring responsible adoption.

Real-time claims adjudication (RTA) enables immediate, transparent, and accurate determination of patient responsibility and provider payment at the point of care, eliminating the weeks-long delay inherent in today's claims cycle. In the current system, patients often wait weeks after a visit to understand their financial responsibility, and providers experience substantial cash-flow delays due to slow, manual, and administratively heavy claims review processes; problems driven by inconsistent data, complex coding requirements, and payer-imposed friction that can reduce or delay payment.⁹ HHS adopting real-time adjudication replaces this opaque and costly model by processing claims instantly, allowing members to know what they owe before or during the visit and enabling providers, including those treating Medicare patients, to be paid immediately at the point of service. Externally, research shows that real-time adjudication can dramatically reduce administrative burden, standardize coding and claims processes, cut errors, improve transparency, and address coordination failures across health systems.⁴ When paired with AI, such as automated coding validation, fraud detection, and real-time policy checks, RTA can further lower denial rates, reduce administrative overhead, and accelerate clean claims, offering a foundation for new incentive models where faster payments can be exchanged for lower rates, aligning payer-provider interests and improving system-wide efficiency.

To amplify the value of the AI use cases outlined above, HHS can implement a federated system that enables agencies and institutions to collaborate without pooling sensitive data. A federated learning approach delivers

privacy and collaboration to accelerate HHS impact. Federated Learning is an approach to train AI/ML models over decentralized data. It is accomplished by distributing a model and its corresponding training code to a decentralized set of edge nodes that hold local datasets, execute the code to train the model on the local data, and return the trained model to a central orchestrator. The orchestrator then assembles all locally trained models into a global model. This means that participating organizations can keep their data secure behind their firewalls, while permitting other users to use their data for training purposes. By keeping data securely within its original environment, federated learning enhances privacy, reduces transfer and infrastructure costs, and supports robust, uncompromised collaboration at scale. When implemented across agencies with clear data standards and effective governance, federated learning can empower HHS to rapidly develop AI models that can unlock breakthroughs in research across chronic disease, nutrition, and other public health threats. Federated learning also enables quick wins for HHS across automation of research workflows and accelerates research breakthroughs in areas such as chronic disease management, nationwide outbreak detection, management of multi-center clinical trial augmentation, and discovery of genetic/rare diseases.¹⁰

HHS can set standards for data use in federated learning.⁸

- **Childhood Autism Research:** With similar challenges as health research (e.g. data silos, fragmented datasets located across hospitals, schools, therapy centers, etc.), a federated system can support autism research by providing early diagnostic prediction models, improved understanding of heterogeneity in autism symptoms and characteristics, and more accurate matching of interventions to patient profiles.
- **Diabetes Research:** Datasets that inform chronic disease cannot be readily pooled, but with federated learning, HHS could identify progression models for Type 2 Diabetes based on diet, activity level, medication adherence, social determinants of health, and glucose levels, among other factors.
- **Vaccine Safety:** Vaccine safety monitoring relies on vast and diverse datasets (e.g. clinical trial results, electronic health records, surveillance data) which are held by different agencies. HHS can coordinate the analysis of trusted and verified data to maintain the highest levels of understanding for vaccine safety.

Deloitte welcomes the opportunity to support HHS as it considers how to best modernize data, policy, and regulatory infrastructure to responsibly accelerate the adoption of AI across health systems. Our work across payers, providers, life sciences, public health, and federal health agencies gives us a uniquely crosscutting perspective on both the operational realities and the policy levers that can unlock meaningful, near-term impact. Our interdisciplinary team would be pleased to discuss these ideas further and partner with HHS to explore pragmatic, scalable solutions that advance transparency, strengthen interoperability, and ensure AI is deployed safely and effectively in the American healthcare system.

Table: RFI Question and Answer Crosswalk

#	RFI Question	Reference
1	What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?	Page 1
2	What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why?	Page 2
2	What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations	Page 1
3	For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them	Page 1
4	For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care?	Page 2
4	Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?	Page 1
6	What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs	Page 2
7	What are the primary administrative hurdles to the adoption of AI in clinical care?	Page 1
8	Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.	Page 2
9	What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?	Page 2
10	Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?	Page 1
10a	a. Are there published findings about the impact of adopted AI tools and their use in clinical care?	Page 1
10b	How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?	Page 1

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